

University Cancer Institute - Long-Term Survivor Clinic
OUTPATIENT PROGRESS NOTE

Patient: Stewart, Jerry

MRN: AML055

DOB: 27-Jan-1986

Date of Visit: 20-Mar-2024

SUBJECTIVE:

Mr. Stewart is a 38-year-old male with a history of favorable-risk AML (inv(16)), now 16 months post-completion of all therapy. He comes in for his routine surveillance visit. He reports feeling "100% back to normal." He is working full-time as a physical therapist and has resumed his regular exercise routine, including marathon training. He reports excellent energy and stamina. He denies any fevers, chills, night sweats, new bruising, or bleeding. He notes some very mild, residual tingling in his fingertips, which is a long-standing issue since his HiDAC therapy, but states it does not interfere with his daily life or work and is stable.

TREATMENT HISTORY RECAP:

- **Diagnosis:** AML w/ inv(16)(p13.1q22) on 19-Nov-2022.
- **Induction:** Standard "7+3" (Daunorubicin/Cytarabine) started on 22-Nov-2022. Achieved a complete remission.
- **Consolidation:** Completed four cycles of high-dose cytarabine (HiDAC), with the final cycle finishing in May 2023.

OBJECTIVE:

Vitals: T 36.7°C, HR 58, BP 115/70 mmHg.

Weight: 88 kg (stable).

General: Appears fit and healthy, in no acute distress.

Exam: Entirely unremarkable. No lymphadenopathy or hepatosplenomegaly.

Neurologic exam reveals 5/5 strength throughout; sensation is intact to light touch, though he reports subjective paresthesias in distal fingertips.

Labs (20-Mar-2024):

- **CBC:** WBC 6.8 K/uL, ANC 4.2 K/uL, Hgb 15.1 g/dL, Plt 295 K/uL.
- **CMP:** All within normal limits.

ASSESSMENT & PLAN:

Mr. Stewart is a 38-year-old male with ELN 2022 favorable-risk AML who remains in a durable complete remission. His clinical status is excellent.

1. **AML Status:** No evidence of disease. Given his favorable cytogenetics and sustained remission well over a year from therapy, his risk of relapse is now quite low. His prognosis is excellent. We will continue surveillance as per protocol.

- **Plan:** Continue with clinic follow-up every 4 months for the next year. He is scheduled for his 2-year surveillance bone marrow biopsy in November 2024.
- 2. **Chemotherapy-Induced Peripheral Neuropathy:** Mild, stable, and non-bothersome. This is a very common long-term side effect of high-dose cytarabine. It is unlikely to progress now that he is far out from treatment.
 - **Plan:** No intervention needed at this time. Advised to report any worsening.
- 3. **Health Maintenance:** Counseled on the importance of long-term health, including diet, exercise, and sun protection. Reviewed his vaccination schedule, and he is up to date on his post-chemo immunizations.
- 4. **Psychosocial:** He is well-adjusted and has a positive outlook. Offered survivorship resources, which he declined at this time.

Overall, this is a best-case scenario follow-up. He can be considered cured, though we use the term "long-term remission" officially.

Dr. Eleanor Vance, MD
Hematology Service